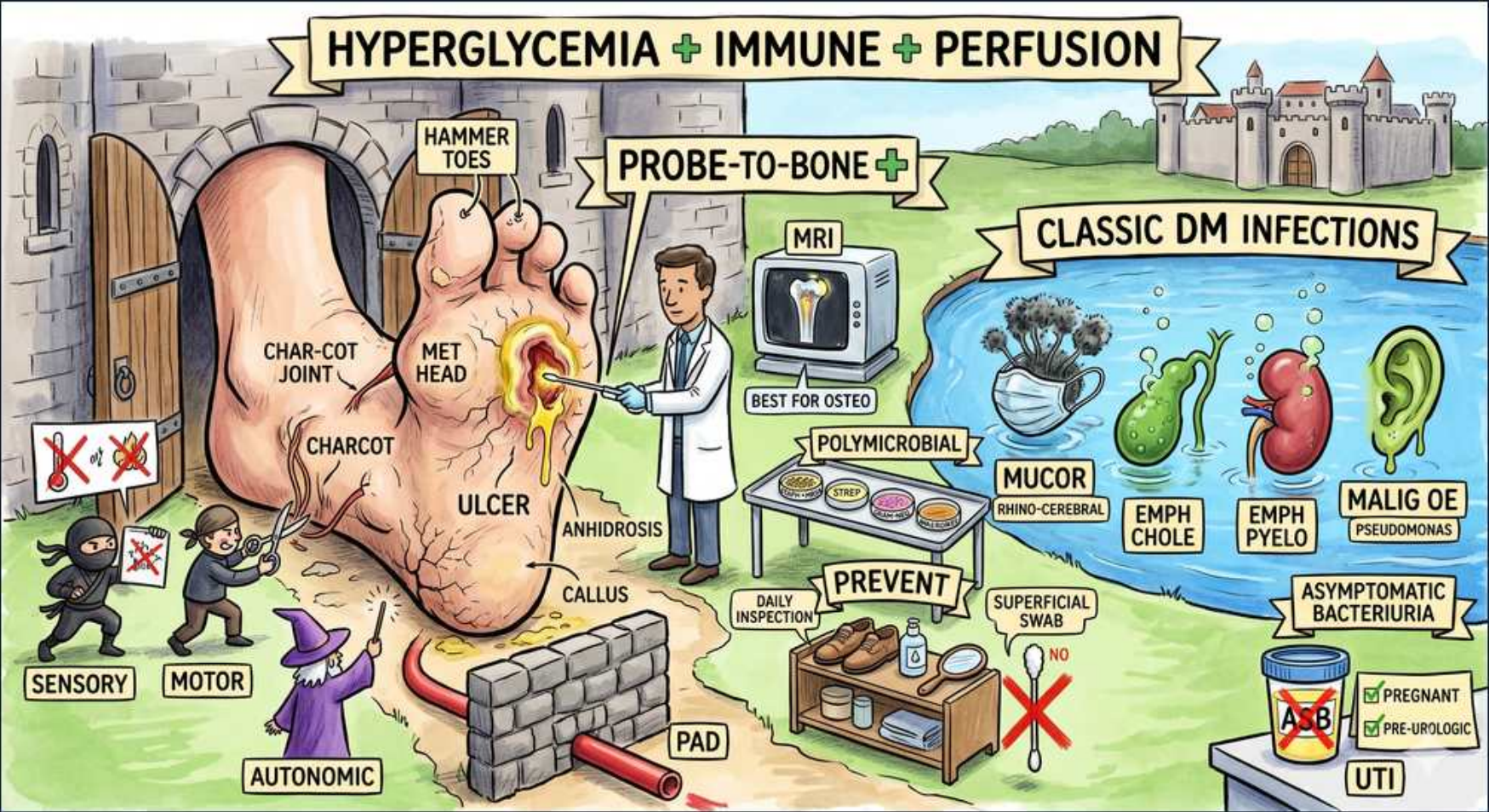




1. Hyperglycemia + immune + perfusion banner

WHAT YOU SEE

Banner HYPERGLYCEMIA + IMMUNE + PERFUSION

WHAT IT ENCODES

Diabetic foot disease arises from a triad: NEUROPATHY (loss of protective sensation → unnoticed trauma), poor PERFUSION (PAD → impaired healing), and impaired IMMUNITY/hyperglycemia (blunted neutrophil function → infection). All three converge to turn minor trauma into non-healing, infected ulcers and amputation. Mnemonic: 'numb foot + no blood + no defense = the diabetic foot.'

BOARD TRAP

WRONG to attribute diabetic foot ulcers to PAD/ischemia alone — the dominant initiating factor is NEUROPATHY (loss of protective sensation). Discriminator: most diabetic foot ulcers are neuropathic (warm foot, palpable or reduced pulses) at pressure points, whereas purely ischemic ulcers are painful, on the toes/margins, with absent pulses.

2. Sensory ninja

WHAT YOU SEE

A black-clad ninja labeled SENSORY creeping up unseen — the stealthy attacker = silent loss of protective sensation that lets trauma go unfelt.

WHAT IT ENCODES

Sensory neuropathy causes LOSS OF PROTECTIVE SENSATION in a stocking-glove distribution, so repetitive trauma, foreign bodies, and pressure go unnoticed until an ulcer forms — the dominant mechanism of diabetic foot ulceration. Screen annually with the 10-g monofilament plus a 128-Hz tuning fork (vibration). Mnemonic: 'can't feel it = can't protect it.'

BOARD TRAP

WRONG to rely on the patient reporting pain or symptoms to detect the at-risk foot — loss of protective sensation is usually ASYMPTOMATIC. Discriminator: an insensate foot on monofilament testing identifies ulcer risk even when the patient feels 'fine,' so screening (not symptoms) drives prevention.

3. Motor ninja / hammer toes

WHAT YOU SEE

Ninja labeled MOTOR with HAMMER TOES above

WHAT IT ENCODES

Motor neuropathy weakens the intrinsic foot muscles, causing imbalance that produces deformities — hammer/claw toes, high arch, prominent metatarsal heads — which create new abnormal PRESSURE POINTS where neuropathic ulcers form under callus. Mnemonic: 'wasted intrinsics → clawed toes → new pressure ulcers.'

BOARD TRAP

WRONG to treat hammer/claw-toe deformities as purely orthopedic/cosmetic — in a diabetic they create high-pressure sites for ulceration. Discriminator: deformity-driven pressure points warrant offloading and protective/custom footwear, not just observation, because they predict ulcer location.

4. Autonomic ninja / anhidrosis

WHAT YOU SEE

Ninja labeled AUTONOMIC with ANHIDROSIS label on foot

WHAT IT ENCODES

Autonomic neuropathy in the foot causes ANHIDROSIS (loss of sweating) → dry, cracked, fissured skin that becomes a portal of entry for bacteria. It also opens arteriovenous shunts, giving a warm, often bounding-pulse foot despite microvascular dysfunction. Manage with emollients and meticulous skin/foot care. Mnemonic: 'dry, cracked, warm autonomic foot lets infection in.'

BOARD TRAP

WRONG to interpret a warm foot with palpable/bounding pulses as proof of good tissue perfusion — autonomic AV shunting can mask significant PAD. Discriminator: still assess ABI/pulses and dryness/fissuring; warmth from autonomic shunting does not exclude underlying ischemia or infection risk.

5. Charcot joint

WHAT YOU SEE

Foot deformity labeled CHARCOT JOINT, CHARCOT

WHAT IT ENCODES

Charcot neuroarthropathy = progressive bone/joint destruction in an insensate foot from repetitive unperceived trauma, producing a warm, swollen, erythematous, often deformed 'rocker-bottom' midfoot. Acute Charcot can closely MIMIC infection. Treatment is prolonged OFFLOADING (total-contact cast) and immobilization. Mnemonic: 'hot swollen deformed foot with intact skin = think Charcot.'

BOARD TRAP

WRONG to treat a red, hot, swollen diabetic foot WITHOUT an ulcer or break in the skin as cellulitis/osteomyelitis and start antibiotics — acute Charcot is the key mimic. Discriminator: no skin breach, midfoot collapse/'rocker-bottom,' and often improvement when the limb is elevated favor Charcot over infection; X-ray/MRI and clinical context distinguish them.

6. Met head pressure / ulcer / callus

WHAT YOU SEE

Plantar ulcer labeled MET HEAD, ULCER, CALLUS

WHAT IT ENCODES

Neuropathic ulcers classically form at plantar PRESSURE POINTS — most often under the METATARSAL HEADS — where callus builds up over the high-pressure area and underlying tissue breaks down. They are typically painless, surrounded by callus, with a 'punched-out' appearance. Treatment centers on OFFLOADING (total-contact cast), debridement of callus, and glycemic control. Mnemonic: 'callus over the met head hides the neuropathic ulcer.'

BOARD TRAP

WRONG to debride callus/ulcer and dress it while leaving the patient weight-bearing — without OFFLOADING the ulcer will not heal. Discriminator: the single most important intervention for a plantar neuropathic ulcer is pressure offloading (total-contact cast), not just topical wound care or antibiotics.

7. PAD wall

WHAT YOU SEE

Crumbling wall labeled PAD

WHAT IT ENCODES

Peripheral arterial disease impairs perfusion and wound HEALING and raises amputation risk; diabetic PAD tends to be distal (tibial/peroneal) and is often clinically silent due to coexisting neuropathy. Assess pulses and the ankle-brachial index (ABI) in any foot ulcer; calcified non-compressible vessels can falsely ELEVATE the ABI (>1.3), so use toe-brachial index or waveforms if suspected. Revascularize ischemic, non-healing wounds. Mnemonic: 'no blood, no healing — check the ABI.'

BOARD TRAP

WRONG to accept a 'normal' or high ABI as ruling out ischemia in a diabetic — medial arterial calcification makes vessels non-compressible and falsely elevates the ABI. Discriminator: an ABI >1.3 (or non-healing ulcer with a 'normal' ABI) should prompt toe pressures/waveform studies rather than reassurance.

8. Probe-to-bone

WHAT YOU SEE

Doctor probing labeled PROBE-TO-BONE

WHAT IT ENCODES

A positive PROBE-TO-BONE test (a sterile blunt probe reaches bone through the ulcer) strongly raises the likelihood of underlying OSTEOMYELITIS, especially in deep or long-standing ulcers; combined with an elevated ESR/CRP it is a useful bedside predictor. Confirm with MRI and bone culture/biopsy. Mnemonic: 'touch bone, suspect bone infection.'

BOARD TRAP

WRONG to rule OUT osteomyelitis just because a plain X-ray is normal — early osteomyelitis is X-ray-occult for ~2 weeks. Discriminator: a positive probe-to-bone, ulcer >2 cm/>1 cm deep, or a markedly elevated ESR should prompt MRI and bone biopsy rather than relying on a normal radiograph.

9. MRI best for osteo

WHAT YOU SEE

Scanner labeled MRI, BEST FOR OSTEO

WHAT IT ENCODES

MRI is the most sensitive and specific IMAGING for diabetic foot OSTEOMYELITIS (detects marrow edema before bone destruction and defines abscess/soft-tissue extent). Plain X-ray is the initial test but is insensitive early; the definitive diagnosis is BONE biopsy with histology and culture to guide targeted antibiotics. Mnemonic: 'X-ray first, MRI to confirm, bone biopsy to prove.'

BOARD TRAP

WRONG to choose plain X-ray as the best test for early osteomyelitis — it is insensitive for the first ~2 weeks. Discriminator: MRI is the imaging of choice; when MRI is contraindicated, use a tagged WBC scan, but the gold standard remains bone biopsy/culture.

10. Polymicrobial / culture

WHAT YOU SEE

Table labeled POLYMICROBIAL, with SUPERFICIAL SWAB crossed out

WHAT IT ENCODES

Chronic/deep diabetic foot infections are often POLYMICROBIAL (Gram-positive cocci like S. aureus/streptococci plus Gram-negatives and anaerobes); MRSA and Pseudomonas matter with prior antibiotics or maceration. Obtain DEEP tissue or bone cultures (after debridement) — NOT superficial swabs — to guide therapy; empiric coverage is broad then narrowed. Mnemonic: 'deep cultures for polymicrobial feet.'

BOARD TRAP

A superficial wound SWAB is the WRONG specimen — it grows surface colonizers and misleads therapy. Discriminator: send DEEP tissue or bone culture (and treat based on it), rather than tailoring antibiotics to a superficial swab result.

11. Prevent (daily inspection / footwear)

WHAT YOU SEE

Shoes/sign labeled PREVENT, DAILY INSPECTION

WHAT IT ENCODES

Prevention is the highest-yield intervention: DAILY foot self-inspection (use a mirror), proper protective/therapeutic footwear and custom orthotics for high-risk feet, regular podiatry/callus care, smoking cessation, and glycemic control. Annual comprehensive foot exams stratify risk; the strongest predictor of a future ulcer is a PRIOR ulcer. Mnemonic: 'look every day, the right shoes every step.'

BOARD TRAP

WRONG to defer foot screening to only symptomatic patients — neuropathy is silent and prior ulceration predicts recurrence. Discriminator: high-risk feet (neuropathy, PAD, deformity, prior ulcer) need structured prevention and protective footwear, not reactive care only after a wound appears.

12. Mucor (rhino-cerebral)

WHAT YOU SEE

Mold jar labeled MUCOR, RHINO-CEREBRAL

WHAT IT ENCODES

Rhinocerebral mucormycosis (Mucor/Rhizopus) classically strikes patients in DKA/uncontrolled diabetes — acidosis and high glucose free up iron and impair phagocytes. It presents with facial pain/swelling, black necrotic nasal/palatal eschar, and rapid orbital/cerebral invasion. Emergency treatment is aggressive surgical DEBRIDEMENT plus IV liposomal amphotericin B and correction of the acidosis. Mnemonic: 'DKA + black eschar = Mucor — cut and amphi now.'

BOARD TRAP

WRONG to start antibacterials and observe a diabetic with facial/sinus pain and a black nasal eschar — this is a surgical fungal emergency. Discriminator: black necrotic eschar with non-septate, broad, ribbon-like, right-angle-branching hyphae in a DKA patient is mucormycosis (needs urgent surgery + amphotericin), not bacterial sinusitis; Aspergillus, by contrast, has septate acute-angle hyphae.

13. Emphysematous cholecystitis / pyelonephritis

WHAT YOU SEE

Gallbladder + kidney labeled EMPH CHOLE, EMPH PYELO

WHAT IT ENCODES

Diabetics are prone to gas-forming (EMPHYSEMATOUS) infections: emphysematous cholecystitis (often Clostridium/E. coli, frequently acalculous, high gangrene/perforation risk → urgent cholecystectomy) and emphysematous pyelonephritis (gas in renal parenchyma, usually E. coli/Klebsiella → broad antibiotics, drainage, sometimes nephrectomy). Mnemonic: 'gas in the gallbladder or kidney = think diabetic + go aggressive.'

BOARD TRAP

WRONG to manage emphysematous cholecystitis/pyelonephritis conservatively like routine cholecystitis/UTI — they are life-threatening, gas-forming infections needing urgent source control. Discriminator: GAS in the organ wall/parenchyma on imaging mandates emergent surgery/drainage plus broad-spectrum antibiotics, not antibiotics alone.

14. Malignant otitis externa

WHAT YOU SEE

Ear labeled MALIG OE, PSEUDOMONAS

WHAT IT ENCODES

Malignant (necrotizing) otitis externa is a PSEUDOMONAS aeruginosa infection of the external canal/skull base in ELDERLY DIABETICS, presenting with severe ear pain (often out of proportion, worse at night), otorrhea, and granulation tissue at the bony-cartilaginous junction; it can extend to skull-base osteomyelitis and cranial nerve palsies (especially CN VII). Treat with prolonged anti-pseudomonal therapy (e.g., ciprofloxacin) ± surgical debridement. Mnemonic: 'elderly diabetic + severe ear pain + granulation = Pseudomonas skull base.'

BOARD TRAP

WRONG to treat severe ear pain with granulation tissue in an elderly diabetic as ordinary otitis externa with topical drops — this is a skull-base osteomyelitis emergency. Discriminator: pain out of proportion, granulation at the canal floor, and an elevated ESR in a diabetic warrant CT/MRI and prolonged systemic anti-pseudomonal antibiotics, not topical therapy alone.

15. Asymptomatic bacteriuria sign

WHAT YOU SEE

Urine cup labeled ASB, UTI, with PREGNANT and PRE-UROLOGIC, NO treat

WHAT IT ENCODES

Asymptomatic bacteriuria (positive culture without urinary symptoms) should NOT be treated in diabetics — antibiotics do not reduce complications and promote resistance. The ONLY routine indications to treat asymptomatic bacteriuria are PREGNANCY and BEFORE a urologic procedure with anticipated mucosal bleeding. Mnemonic: 'no symptoms, no treatment — except pregnancy or pre-urologic.'

BOARD TRAP

WRONG to treat asymptomatic bacteriuria in a diabetic 'because diabetes' — diabetes is NOT an indication to treat. Discriminator: treat only symptomatic UTIs (or asymptomatic bacteriuria in pregnancy / before urologic procedures); a positive culture without symptoms in a non-pregnant diabetic should be left alone.